

Public Health Emergency Operations Center

« COUSP RDC »

MVE-17 Incident Management System

Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°061/MVB_14/07/2026

Affected provinces (5)	HUT-UELE, ITURI, NORTH-KIVU, SOUTH-KIVU & TSHOPO
Affected Health Zones (45)	• Upper Uele (4/13): Wamba, Boma Mangbetu, Pawa and Isiro • Ituri (26/36) : Aru, Aungba, Bambu, Bunia, Damascus, Drodro, Gety, Kilo, Commander, Lita, Nyankunde, Rimba, Rwampara, Tchomia, Kambala, Nia-Nia, Fataki, Mandima and Ariwara. • North Kivu (11/34) : Beni, Butembo, Goma, Kalunguta, Katwa, Kyondo, Oicha, Masereka, Vuhovi, Mabalako et al The museums. • Sud-Kivu (1/34) : Miti-Murhesa • Shop (3/23): Makiso-Kisangani, Lubunga and Mangobo
Date the report	July 14, 2026
Publication date	July 15, 2026

KEY INDICATORS — CUMULATED AS OF JULY 14, 2026

 CONFIRMED CASES — 5 PROVINCES 2 073 * +62 new cases (Ituri 55 · N-Kivu 7)	 CONFIRMED DEATHS · FATE 796 · (38,4 %)** +42 deaths (Ituri 37 · North Kivu 5)	 SUSPECTED CASES OF THE DAY 418 Ituri 318 · N-Kivu 79 · Shops 10 · Haut-Uele 11
 PATIENTS IN ISOLATION 737 Ituri 544 · N-Kivu 188 · Shop	 CURED — CUMULATIVE 377 +13 recovered (Ituri) · Ituri total 341 · N-Kivu 36	 CONTACT TRACKING — NATIONAL 76,6 % 9 303 / 12 190 vus · Ituri 73.4 % · N-Kivu 92.0 % · Tshopo 62.5 %

* National cumulative total as of 14/07: 2,011 (13/07) + 62 new confirmations = 2,073 cases; 754 + 42 deaths = 796.

** Overall lethality 38.4% (796 / 2073): 35.9% in Ituri, 58.7% in North Kivu, 92.9% in Haut-Uélé, 33.3% in South Kivu and 75% in Tshopo.

RECONCILIATION NOTE — ADDITIONAL TOTALS

The national totals result from adding the new confirmations and deaths of the day to the validated totals from July 13th: 2,011 + 62 = 2,073 cases and 754 + 42 = 796 deaths. North Kivu: 182 + 7 = 189 cases and 106 + 5 = 111 deaths (including 1 at the Oicha Ebola Treatment Center).

1. Epidemiological Highlights

📍 **Ituri : 55 new confirmed cases** (Bunia: 22, Nizi: 8, Rwampara: 7, Mongbwalu: 6, Lita: 5, Nia-Nia: 5, Fataki: 1, Mandima: 1) and 37 deaths (23 community cases, 14 in Ebola Treatment Centers/Treatment Centers); 13 new recoveries. The Djugu-Irumu corridor remains very active.

📍 **North Kivu: 7 new confirmations** (Butembo: 3, Katwa: 2, Musienene: 2) and 5 deaths (including 1 at the Oicha CTE)
; provincial cumulative total 189 cases / 111 deaths (case fatality rate 58.7%), 36 recovered and 20 active cases.

📍 **Haut-Uélé: 0 confirmed cases.** Cumulative total remains at **14 confirmed cases and 13 deaths** (Wamba 8, Pawa 3, Boma Mangbetu 2, Isiro 1); 4 health zones affected.

¶ **Tshopo and South Kivu:** No new confirmation in Tshopo: a confirmed case who escaped from Nia-Nia (Ituri) was found in Lubunga and isolated at the Cinquantenaire CTE; a suspect who came from Nia-Nia was isolated and taken into custody. Training began for 50 healthcare providers and 2.5 tons of IPC supplies were donated to the Provincial Health Division (DPS); construction began on the Kasenyi Ebola Treatment Center (80 beds, Ugandan support). A total of 45 health zones out of the 140 in four (4) Provincial Health Divisions (DPS) are currently affected (Ituri: 26/36, North Kivu: 11/34, Tshopo: 3/23 and Haut-Uélé: 4/13) and 1 health zone (Miti-Murhesa) in South Kivu, which has gone 49 days without a new confirmed case. The province is maintaining residual surveillance in preparation for declaring the end of the epidemic.

Transmission remains intense in the Ituri province, confirming it as the main focus of the epidemic.

The appearance of new cases in Haut-Uélé reflects a geographical expansion requiring an immediate strengthening of surveillance, diagnosis and operational preparedness.

EPIDEMIOLOGICAL AND OPERATIONAL CONTEXT

Declared on **May 15, 2026**, the Ebola virus disease outbreak caused by **Bundibugyo ebolavirus** continues to evolve within a complex humanitarian and security context. Initially detected in the **Mongbwalu Health Zone (Ituri)**, it has spread to **five provinces (Ituri, North Kivu, South Kivu, Haut-Uélé, and Tshopo)**, affecting **45 health zones**. Population movements, insecurity, artisanal mining activities, and cross-border trade with Uganda and South Sudan are facilitating transmission and complicating response operations. Multisectoral coordination continues to limit the spread of the epidemic and strengthen public health interventions.

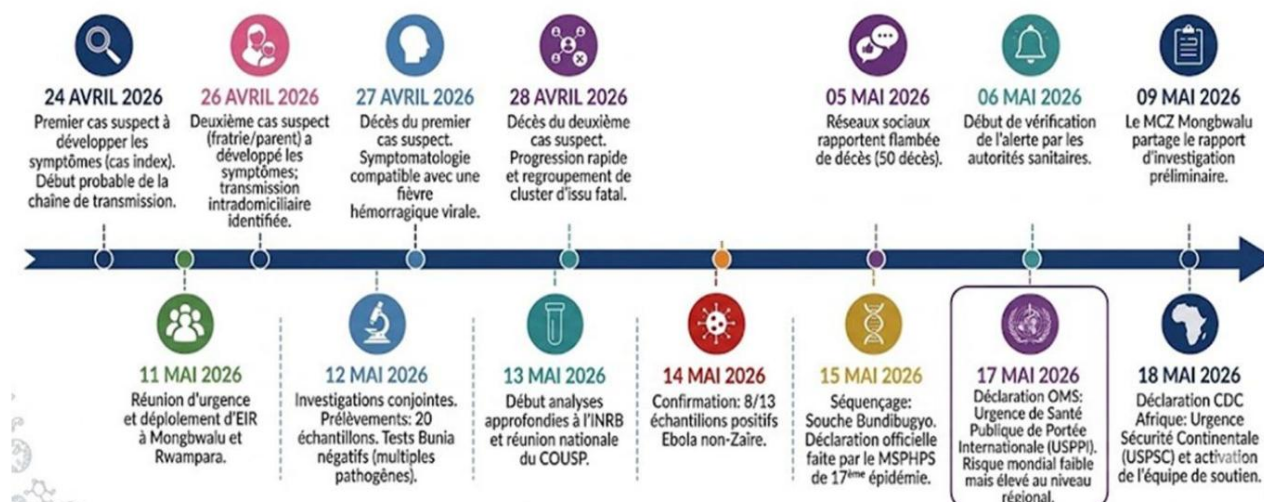


Figure 1. Chronology of events of the Ebola-Bundibugyo virus disease epidemic

2. Descriptive Epidemiological Analysis

¶ 2.1 — Distribution by province and active households

TABLE 1 — DISTRIBUTION OF CONFIRMED CASES AND DEATHS BY PROVINCE

Province	Confirmed cases	Death	Case fatality rate, affected health zones, new cases (24 h)		
Ituri	1 863	668	35,9 %	26 / 36 (72,2 %)	55
North Kivu	189	111	58,7 %	11 / 34 (32,4 %)	7
South Kivu	3	1	33,3 %	1 / 34 (2,9 %)	0
Haut-Uélé	14	13	92,9 %	4 / 13 (30,8 %)	0
Boss	4	3	75,0 %	3 / 23 (13,0 %)	0
Total	2 073	796	38,4 %	45 out of 140	62

Provincial dynamics and case concentration

Ituri province accounts for **89.9% of cases (n=1,863)** and 83.9% of deaths (n=668), resulting in a crude case fatality rate of 35.9%, remaining the epicenter of the epidemic. The health zones of **Bunia (556 cases), Rwampara (412), Mongbwalu (342), Nizi (139), and Nyankunde (99)** remain the most affected. Within the affected provinces, the new cases confirmed as of July 14th are distributed across the health zones of Bunia (22), Nizi (8), Rwampara (7), Mongbwalu (6), Lita (5), Nia-Nia (5), Butembo (3), Katwa (2), Musienene (2), Fataki (1), and Mandima (1).

Severity of the epidemic in North Kivu

As of July 14, North Kivu had recorded **189 confirmed cases and 111 deaths**, representing a **crude case fatality rate of 58.7 %**. The highest number of reported cases was in Katwa (n=78), Butembo (n=49), and Beni (n=31); there were 36 recoveries and 20 active cases. The case fatality rate remains particularly high. in North Kivu must be interpreted with caution in the context of an evolving epidemic, but remains compatible with late detection of cases and underreporting of less severe forms.

Geographic extension — Haut-Uélé and Tshopo

Haut **-Uélé** has risen to **14 confirmed cases** (Wamba 8, Pawa 3, Boma Mangbetu 2, Isiro 1) **and 13 deaths**. As the report for July 14th has not yet been received, these figures remain unchanged from July 13th. Four health zones are affected in the Haut-Uélé health district, while Aba and Doruma remain on alert. Meanwhile, **Tshopo** remains at three health zones (Makiso-Kisangani, Mangobo, Lubunga; 4 cases, 3 deaths) and has not recorded any new confirmed cases. A confirmed case who escaped from the Nia-Nia health zone (Ituri) was found in the Lubunga health zone (Tshopo) and isolated at the Cinquantenaire Treatment Center. All these cases are linked to the epicenter in Ituri (Nia-Nia / PK51). The proximity to Uganda via the Aru health zones Ariwara increases the risk of expansion.

2.2 — Distribution by health zone

TABLE 2 — CONFIRMED CASES AND DEATHS BY PROVINCE AND HEALTH ZONE (CUMULATION AND DAYLY SITUATION)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New case	Community death res (Swab+).	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	1 863	668	35,9 % 55		23	14	37
Bunia	556	185	33,3 % 22		9	2	11
Rwampara	412	101	24,5 %	7	3	5	8
Mongbwalu	342	188	55,0 %	6	2	4	6
Nizi	139	58	41,7 %	8	7	1	8
Legumes	99	26	26,3 %				
Mangala	50	29	58,0 %				
Liter	64	23	35,9 %	5			0
Nanny	41	9	22,0 %	5	1	1	2
Bamboo	31	10	32,3 %				
Team	27	10	37,0 %				
Tchomia	19	4	21,1 %				
Unidentified	17	0	0,0 %				
Kilo	9	1	11,1 %				
Darkness	9	4	44,4 %	1		1	1
Logo	7	0	0,0 %				
Aungba	6	2	33,3 %				
Ladies	5	0	0,0 %				
Washing	5	4	80,0 %				
Jungle	4	0	0,0 %				

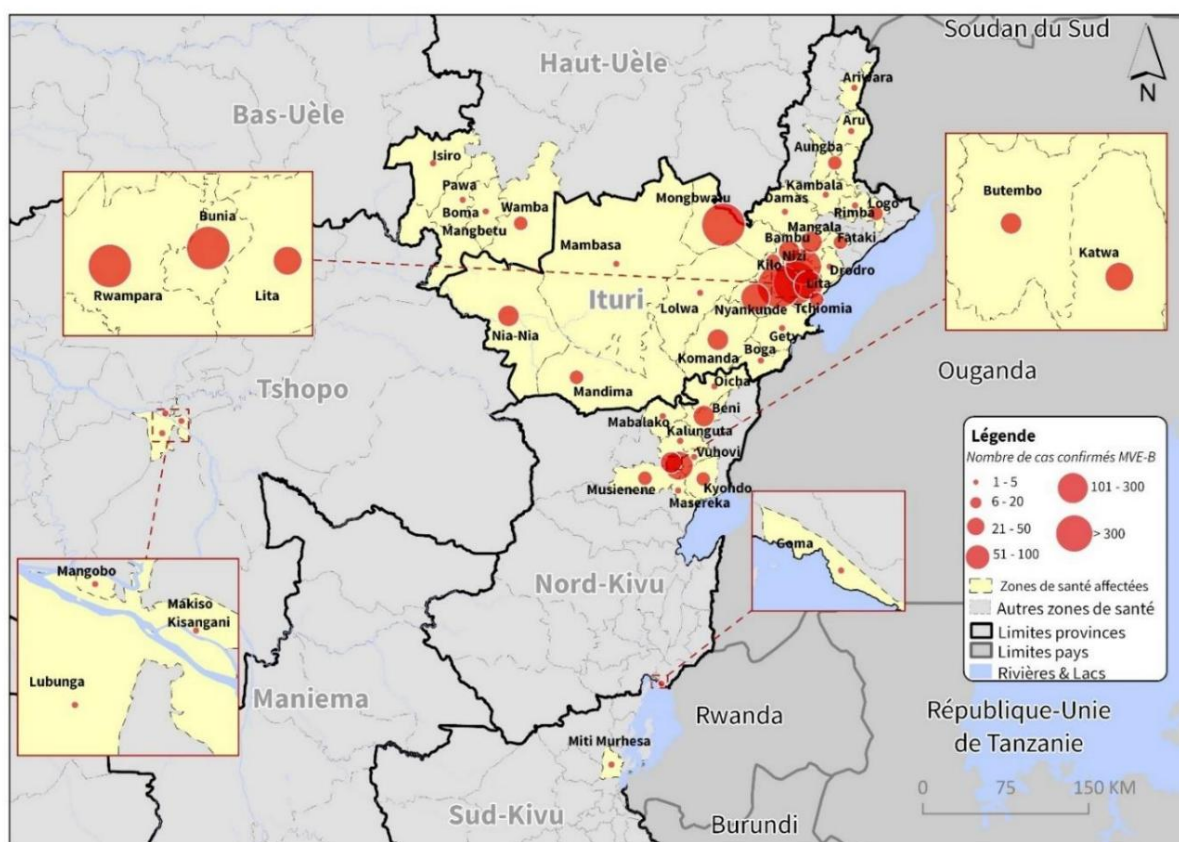
Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Cas (n)	Deaths (n)	Lethality	New case	Community death res (Swab+).	Confirmed deaths within the CTE	Total confirmed deaths
Fireworks	6	5	83,3 %	1	1		1
Drodro	5	3	60,0 %				
Understand	3	1	33,3 %				
Flounder	2	2	100,0 %				
Tide	1	0	0,0 %				
Fight	2	1	50,0 %				
have	1	1	100,0 %				
Ariwara	1	1	100,0 %				
North Kivu	189	111	58,7 % 7		4	1	5
Cut	78	56	71,8 %	2	2		2
Butembo	49	24	49,0 %	3	2		2
Me	31	20	64,5 %				
The museums	13	3	23,1 %	2			
Kyondo	7	3	42,9 %				
Kalunguta	2	1	50,0 %				
Masereka	3	0	0,0 %				
Oicha	3	3	100,0 %			1	1
Ten	1	0	0,0 %				
Vuh	1	1	100,0 %				
Be anxious.	1	0	0,0 %				
South Kivu	3	1	33,3 % —				—
Miti-Murhesa	3	1	33,3 %				
Haut-Uélé	14	13	92,9 % —				—
Ordinary	8	8	100,0 %				
Lining	3	3	100,0 %				
Kill Mangbetu	2	1	50,0 %				
Mathematics	1	1	100,0 %				
Boss	4	3	75,0 % —				—
Makiso-Kisangani	2	2	100,0 %				
Mangobo	1	0	0,0 %				
Lubunga	1	1	100,0 %				
Total	2 073	796	38,4 % 62		27	15	42

The persistent concentration of cases in a limited number of health zones suggests that the current dynamics are primarily driven by a few major transmission hotspots. Targeted intensification of investigations, contact tracing, and community interventions in these areas should have a greater impact than a uniform dispersal of resources across all affected areas.

2.3 — Mapping of confirmed cases

The spatial distribution confirms the concentration of cases in the health zones of — Bunia, Rwampara, Mongbwalu, Nizi, Nyankunde with an extension towards North Kivu (Beni, Butembo, Katwa) and Haut-Uélé.

FIGURE 1 — HEALTH ZONES THAT HAVE REPORTED CONFIRMED CASES (AS OF 07/14/2026)



The recent emergence of new outbreaks in Haut-Uélé confirms a progressive expansion of the affected areas, highlighting the need to simultaneously maintain intensive interventions at the epicenter and preparedness activities in neighboring provinces.

2.4 — Temporal analysis — epidemic curves

We are seeing an increasing number of confirmed cases from one week to the next, reflecting ongoing community transmission. Epidemiological weeks S25, S26, and S27 confirm this continued growth, with more than 300 confirmed cases per week.

FIGURE 2 — EPIDEMIC CURVE BY WEEK OF NOTIFICATION

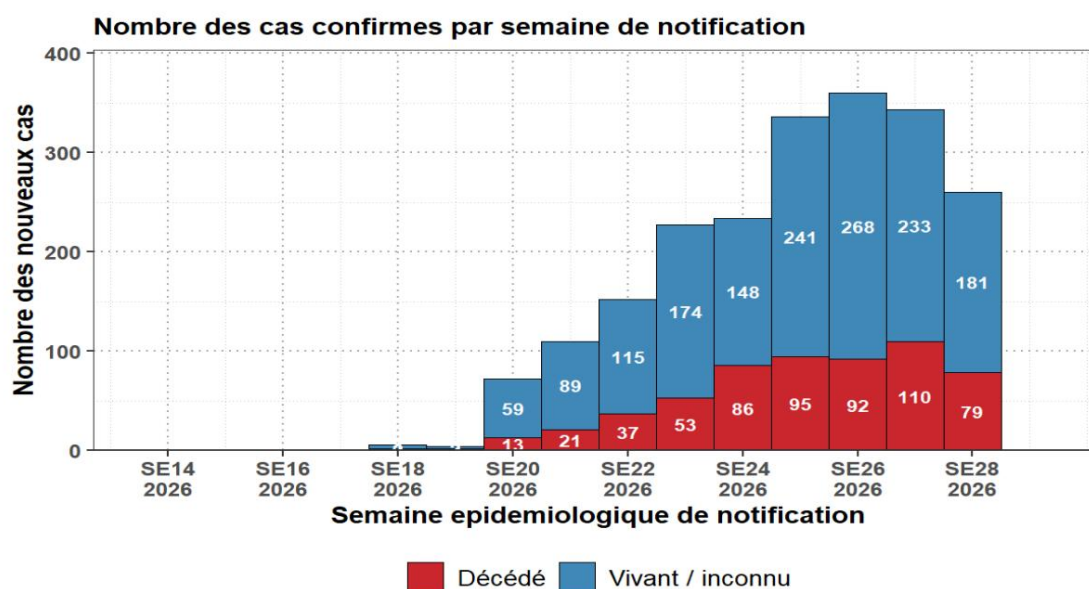
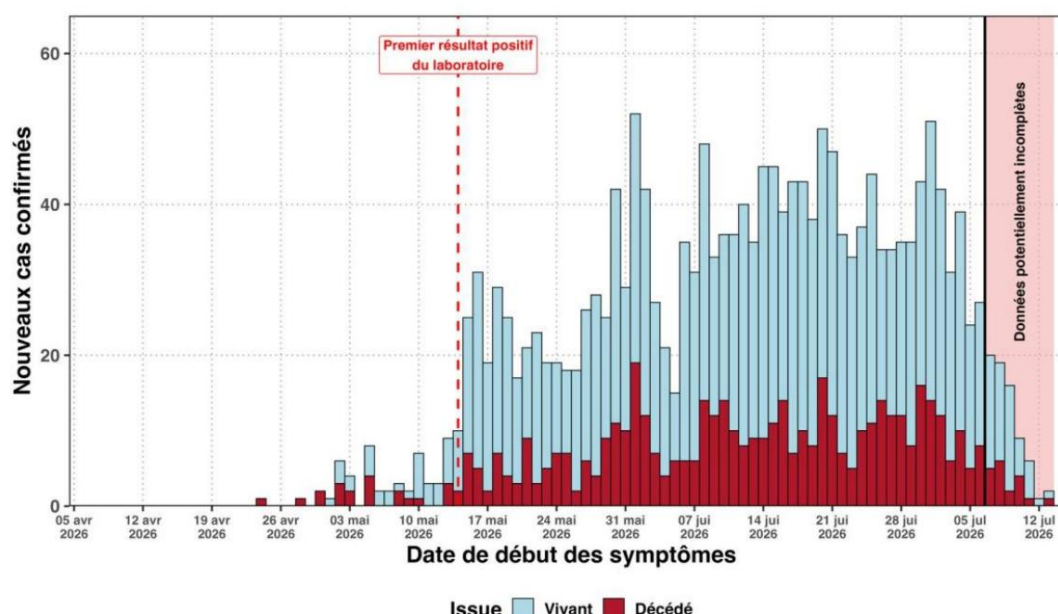


FIGURE 3 — EPIDEMIC CURVE BY DATE OF SYMPTOM ONSET (LINEAR LIST DHIS2)



The epidemic continues a phase of sustained transmission, characterized by a rapid increase in the number of cases since the outbreak began, followed by a period of relative stabilization at a still high level of transmission. Fluctuations observed in recent days should be interpreted with caution due to reporting delays and the ongoing consolidation of data.

2.5 — Demographic characteristics

The age pyramid shows a predominance of **working-age adults** (18–50 years). Minors under 18 and people over 50 are proportionally less affected.

FIGURE 4 — CONFIRMED CASES BY SEX AND AGE GROUP

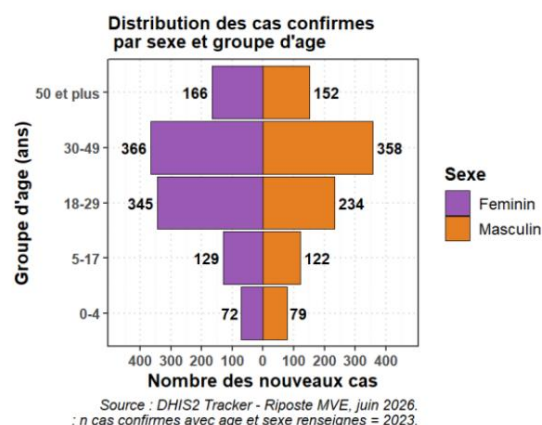
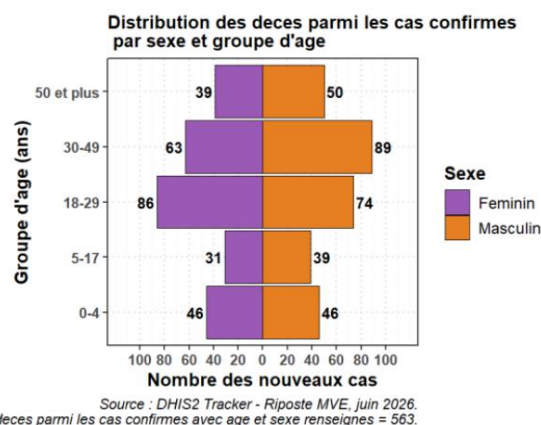


FIGURE 5 — DEATHS BY SEX AND AGE GROUP (AMONG THE (CONFIRMED CASE)



The distribution by sex remains generally balanced, with no major differences suggesting preferential transmission by sex. The observed differences are more likely related to exposure patterns than to different biological susceptibility. The concentration of cases among working-age adults is consistent with more frequent community and occupational exposures. However, the significant proportion of pediatric cases confirms the persistence of intrafamilial transmission in several affected areas.

3 Surveillance, tests et transmission

3.1 — Alert Management and Sources

TABLE 3 — MANAGEMENT OF EPIDEMIOLOGICAL ALERTS AS OF 14/07/2026

Indicator	Ituri	North Kivu	Haut-Uele	Boss	Total
Report (D-1)	7	12.	0	0	19
New alerts received	385	725.	12	10	1132
Total alerts for the day	392	737.	12	10	1151
Alerts investigated	179	725.	12	10	926
Investigation rate (24 h)	45.7%801.79 98.4n.d.95, %		100%	100100 %	80,5%
Alerts confirmed — live	273	52	11	6	342
Alerts confirmed — deceased (comm.)	45	27	0	4	76
Total suspected cases for today	318	79	11	10	418

Ituri: **392 alerts** (385 new), 318 validated, 179 investigated within 24 hours (45.7%); 101 suspected cases isolated out of 134.

North Kivu: 79 **suspected cases** (52 living, 27 community deaths). The reporting of alerts remains low in the provinces of Tshopo, South Kivu and Haut-Uélé.

3.2 — Laboratory: tests and positivity

ŷ **Ituri**: 152 samples analyzed (106 blood, 46 swabs) of which **55 were positive**, representing a positivity rate of 36.2%

ŷ **North Kivu**: 110 samples analyzed, of which **7 were positive** (positivity rate 6.4%).

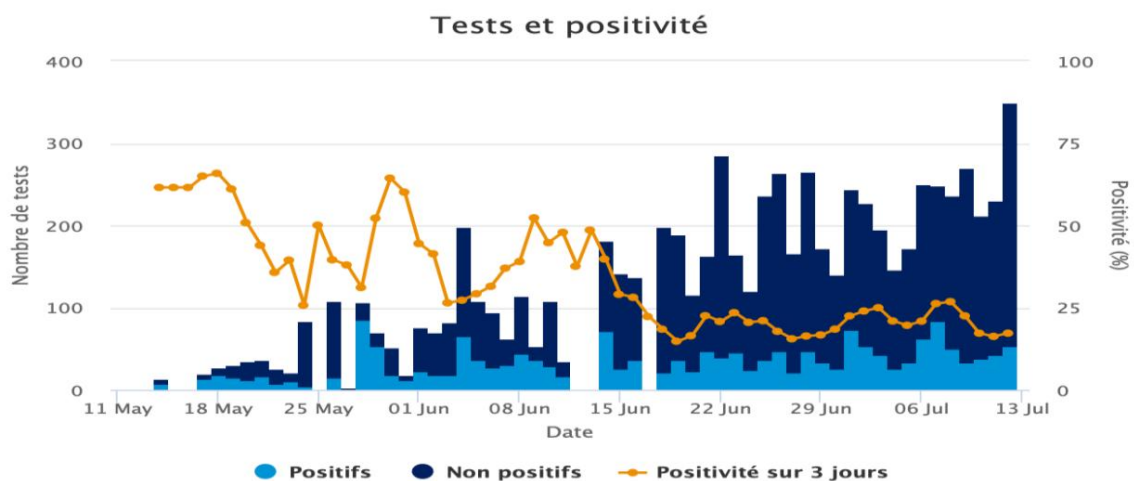
ŷ **South Kivu**: 4 samples collected, currently being analyzed

Haut-Uele : 9 samples collected, currently being analyzed

Tshopo: 1 sample collected, currently being analyzed

Diagnostic capacities remain under considerable strain, particularly in the province of Ituri, which analyzed 152 samples with 55 positive results (positivity rate ŷ 18%). The observed positivity confirms the continuation of active transmission, particularly in Ituri, while maintaining diagnostic delays remains a critical factor for the rapid interruption of transmission chains.

FIGURE 6 — TESTS AND POSITIVITY (COUSP SITUATION REPORT)



N = 7,390 tests. Source : rapport de situation COUSP.

3.3 — Contact Tracking

TABLE 4 — CONTACT TRACKING OF CONFIRMED CASES

Province	Contacts sous follow up	Contacts vus	tracking rate
Ituri	9 731	7 142	73,4 %
North Kivu	2 172	2 001	92,0 %
Boss	147	92	62,5 %
Haut-Uélé	ND	ND	ND
South Kivu (end of monitoring)	0	0	THAT
Total	12 050	9 235	76,6 %

Ituri: 7,142 contacts seen out of 9,731 (73%), 260 new contacts listed, and 118 identified as suspects. Four (4) health zones did not submit their follow-up reports (Ariwara, Nia-Nia, Kambala, Damas). North Kivu: 2,001 contacts seen out of 2,172 (92%). In Tshopo, 92 contacts followed up out of 147 (62.5%, below target), and 20 community health workers were briefed. The overall follow-up rate (76.6%) remains below the recommended operational threshold (95%).

3.4 Entry points and control points (PoE/PoC)

The 71 checkpoints (PoCs) and points of entry (PoEs) remain activated and reinforced (Ituri 57, North Kivu 14). As of July 13, **154,481** travelers had been screened (Ituri 94,294, North Kivu 60,187), with a screening rate of approximately 96% and an awareness rate of 98%. Seven (7) alerts were notified: 6 in Ituri (5 confirmed cases of living individuals referred to the CT) and 1 in North Kivu (1 body intercepted).

3.4.1 — Crossing point alerts (24 h)

Ituri — 6 alerts (5 confirmed active)

At **PoC Ombayi** (2 alerts, one confirmed): a 17-year-old girl (Otsokotsu market ȳ Kaliko, fever/headache) confirmed, isolated and transferred to the CT. At **PoC Makoko 2** (4 alerts): 4 suspected cases (including PK3 Arua 2, PK44ȳMambasa, PK4 Mumbere road) confirmed, isolated and transferred to the transit center.

North Kivu — 1 alert (invalidated)

A live alert was reported and subsequently invalidated after thorough investigation at the **Eringeti PoC**. A field visit was conducted with the Oicha MCZ (Military Coordination Zone) with a view to activating and integrating the Matombo PoC (Oicha–Eringeti axis).

3.4.2 — Monitoring indicators for PoE/PoC

TABLE 5 — MONITORING OF INDICATORS AT POINTS OF ENTRY AND CHECKPOINTS (POE/POC), AS OF 14/07/2026

Indicators	Ituri	North Kivu	Global
Completeness of daily reports	93 % (53/57)	100 % (14/14)	94 % (67/71)
Proportion of PoC enabled and PoE enhanced	100 % (57/57)	100 % (14/14)	100 % (71/71)
Number of people who have switched to PoE/PoC	98 665	58 560	157 225
% of travelers screened % of	96 %	98 %	96 %
travelers who washed their hands % of travelers	97 %	98 %	95 %
made aware	98,3 %	98 %	98,2 %
Number of alerts notified	12	0	12
Number of validated live alerts	8	0	8
Number of bodies recovered today	3	0	3

Indicators	Ituri	North Kivu	Global
Number of problematic contacts intercepted today; % of validated	0	0	0
active alerts referred to CT/CTE	75 %	0	75 %
% of the lifeless bodies swabed	0%	0	0 %
Number of positive results of referred suspected cases	0	0	0

Indicators for the PoE/PoC pillar as of July 14, 2026 (Ituri + North Kivu). Refusal of testing: 3% in Ituri, 2% in North Kivu. No laboratory feedback on referred suspected cases was recorded today.

4. Support

TABLE 6 — OCCUPANCY OF HEALTHCARE FACILITIES (CTE/CT/CI), AS OF 14/07/2026

Indicator	Ituri	North Kivu	Boss	Ensemble
Patients in bed (Day -1)	541	149	4	694
Total admissions (24 h)	80	52	1	133
deceased	18	1	0	19
Non-case	34	12	1	47
Healed	13	2	0	15
Escapes	12	0	0	12
Transferred to the HGR	0	0	0	0
Total sorties (24 h)	77	13	1	91
Patients in isolation (end of day)	544	188	4	736
including those confirmed	249	20	1	270
dont suspects	295	168	3	466
Occupancy rate	78% 129.6% 60.0%	Saturation persists in North		87,4 %

Kivu (129.6%), with a critical shortage of isolation beds; Ituri (78%) remains under pressure. In total, **753 patients are in isolation** (270 confirmed, 466 suspected), with an overall occupancy rate of 87.4%.

5 Mental Health and Psychosocial Support (MHPSS)

TABLE 7 — KEY INDICATORS SMSPS, JULY 14, 2026

SMSPS Key Indicators	Ituri	North Kivu
New confirmed cases that benefited from SMSPS interventions	1 (9,1 %)	1 (100%)
Confirmed cases are being monitored.	78 (55,3 %)	7 (100 %)
New suspected cases	6 (13,6 %)	10 (100 %)
Suspected cases are being monitored	95 (75,4 %)	15 (100 %)
Laboratory results announced (including positive ones)	26(1)	2 (2)
Children separated — nursery / community	5/0	0 / 0
CTE support staff	162	12
Frontline Personnel (PPL)	19	11
Household/Community Members	24	0
EDS supported (SMSPS)	8	0

SMSPS Highlight of 14/07: End of training of service providers (clinical psychologists, APS, TPS, RECO — 27 women, 23 men) of the Nyankunde Health Zone in Ituri.

6 Frontline Personnel (PPL) assigned

TABLE 8 — FRONTLINE STAFF (FLS) INFECTED WITH EVD (CUMULATIVE AS OF JULY 14, 2026, ITURI)

Health Zone	Confirmed cases	Healed	In hospital	Death
Bunia	35	16	5	14
Rwampara	32	23	2	7
Mongbwalu	16	5	3	8
Legumes	14	11	1	2
Nanny	10	0	10	0
Nizi	3	0	0	3
Mangala	3	1	1	1
Bamboo	2	2	0	0
Kilo	2	1	1	0
Liter	2	1	0	1
Total — case fatality rate 31.6%	119	60	25	36

PPL = healthcare workers and frontline providers. Source: Continuity of Essential Services (CSSE) pillar, July 14, 2026. The high infection rate among staff (case fatality rate 30.3%) underscores the need for IPC and PPE.

7. Response activities by pillar — summary as of July 13, 2026

COORDINATION Coordination meeting held with the authorities politico-administrative, sector chiefs and chiefdoms of Ituri, dedicated to their involvement in raising awareness among the population about the Ebola virus disease. Coordination meeting held with the provinces of North Kivu, South Kivu, Ituri, Tshopo, Haut-Uélé and Kinshasa, on the evolution of the Ebola virus disease epidemic in the Democratic Republic of Congo.	PCI / WASH 161/163 ESSs that housed a confirmed case were decontaminated within 48 hours (99%). During the day, 33 out of 51 households with cases were decontaminated (65%). In total, 924 service providers have been trained, but the supply of infection prevention and control (IPC) kits to social and healthcare establishments and households remains low. Tshopo : J 1 of training of 50 ECZ of 4 priority ZS (Makiso Kisangani, Lubanga, Kabondo and Mangobo); Evaluation Scorecard of a 1ESS (University Clinic: 27%) Monitoring and support of healthcare providers; Provision of infection prevention and control (IPC) supplies to EDS teams
Dignified and secure burials (DSM) Ituri: 81 death alerts reported, 57 EDS (Emergency Health Surveys) conducted, and 37 swabs (swabs). Resistance reported in Lopa (Nizi Health Zone) and Rwampara; 14 reports from Nizi remain pending. Training of two EDS teams has begun in Mongbwalu (Yedi and Lodjo Health Zones). North Kivu Report: 11 death alerts out of 27; swabé bodies: 25; non-swabé bodies: 2 (Oicha Health Zone: 1, Beni Health Zone: 1); EDS carried out: 27.	HOLISTIC CARE (HCC) Ituri: Thirteen (13) patients with Ebola virus disease in Ituri province (6 at CTE CME, 3 at CTE Elikya, 3 at CTE ISTM, 1 at CTE Rwampara) have been declared cured after obtaining negative follow-up tests Fourteen (14) deaths were recorded among patients with Ebola virus disease in the CTE/CT in Ituri (4 at CTE Mongbwalu, 3 at CTE Rwampara, 2 at CTE CME, 1 at CTE Elikya, 1 at CT HGR Bunia, 1 at CT Fataki and 1 at CT Nizi) and in North Kivu (1 at CTE Butembo). At the end of the day on July 14, 1305 hot meals were served, including 131 to suspected cases, 141 to confirmed cases, 570 to PPLs and 463 to accompanying persons. Construction has begun on the 80-bed Kasenyi CTE and preparations are underway for a 50-bed CTE in Bambu.

COMMUNICATION (RCCE / CREC) ÿ 19 EDS (Emergency Health Diagnosis) sessions conducted, 9 people engaged through advocacy on the importance of handwashing and how to communicate with passengers, support for community activities (monitoring of community deaths and contacts), referral of patients to health facilities, 399 people reached with the message during home visits (145 men and 254 women) on general information about Ebola virus disease (transmission methods, signs of Ebola virus disease), proper handwashing, and referral of patients to health facilities/Ebola treatment centers 631 people were reached through awareness-raising activities during educational talks held by community actors in the health zone (NYAKUNDE, NIZI, RWAMPARA); 112 people, including village chiefs and leaders of women's associations, were engaged in response activities following community dialogue sessions held in the health zones; 14,174 people were reached through mass awareness-raising sessions held by community actors in the health zone (NYAKUNDE, NIZI, Briefing of 20 local community leaders from PK 51 after the fire at the CTE in the ZS NIA-NIA. Briefing of 12 members of the Mongbwalu Health Zone management team on the various themes (CREC, Community-based Surveillance) Tshopo: Advocacy: Contact with youth leaders and scouts. Training: Support for the training of service providers. MOSO: Continued awareness-raising, dissemination of messages and ads. Support: Assistance with disinfection and case listing.	CONTINUITY OF ESSENTIAL SERVICES (CSSE) ÿ 20/36 ZS of Ituri shared the mapping of partners offering free care; Cap MEG follow-up; follow-up of frontline staff assigned (see Table 8).
RESEARCH, INNOVATION & VACCINATION ÿ Official launch of the EBO-PEP clinical trials; continuation of the Partners vaccine trial and anthropological research in Mongbwalu. Nine protocols have been approved and two of the nine studies are underway or completed.	Logistics ÿ Delivery of 282 packages of PCI/PEC inputs (5,131 kg) to the health zones of Drodro, Mandima and Boga; provision of 25 vehicles and 4 ambulances; delivery of 21 motorcycles to several health zones and supply of COUSP with 140 liters of fuel. Tshopo: Provision of fuel to vehicles for EIR mobility for household disinfection; Reception of the first batch of IPC supplies; Preparation of the tent site for suspected cases; Provision of supplies to IPC monitoring and surveillance teams (contact tracing)
PREVENTION OF SEXUAL EXPLOITATION AND ABUSE (PSEA / PRSEAH) Ituri: 55 people sensitized, bringing the total to 13,836; 2,295 people were briefed and signed the commitment agreement, and 12 CBCMs were revitalized in Mongbwalu.	

In summary, response activities are continuing across all pillars, but several operational constraints continue to limit the speed and effectiveness of interventions in the main transmission hotspots.

8 Main challenges, impacts and actions required

Challenge No.	Impact	Actions required
1 Reluctance to undergo post-mortem sampling (EDS/swabs) and community resistance	Confirmation delays, underestimation of cases, disruption of key activities	Intensify CREC, involve religious and community leaders, secure the teams, increase the number of EDS teams
2 Non-payment of EDS teams (paralysis in Rwampara)	Disruption of dignified and safe burials	Regularize payments to EDS service providers; secure dedicated funding
3 Insufficient capacity of CTE/CT facilities and saturation in North Kivu (129.6%)	Admission delays, increased risk of nosocomial transmission	Accelerate the construction and expansion of CTEs, deploy temporary structures
4 Contact tracing performance not optimal (Haut-Uélé 55.5%)	Uninterrupted transmission chains	Train, deploy and motivate community liaisons (RECOs); enhanced supervision

Challenge No.	Impact	Actions required
5	Ambulance shortages and pre-positioning at points of control	Preposition ambulances (including those at PoE/PoC), deploy CTs per ZS
6	Insufficient supply of MEG and medical inputs	Advocacy and urgent procurement in essential medicines
7	Insufficient PCI inputs (PPE, chlorine) and limited training	Urgent supply, reinforcement of PCI training
8	Geographic extension (Haut-Uélé, Tshopo) and high mobility	Activate the 23 priority PoE/PoC points in Haut-Uélé; strengthen cross-border coordination
9	Installation of local laboratories (Isiro, Mambasa, Logo and Fireworks)	Accelerate the deployment of decentralized diagnostic capabilities
10	Insufficient financial resources	Limitation of the implementation of the pillars
11	Insecurity and limited access (CODECO, ADF)	Strengthening security coordination and humanitarian access
12	Continuity of essential services (CEHS) compromise	Increase in indirect (non-Ebola) mortality
		Effective implementation of free healthcare, strengthening of CEHS

9 Photos of field activities



Community briefing in ZS MONGBWALU



Awareness-raising in ZS LITA

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INFORMATION
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Avec l'appui technique de l'Organisation Mondiale de la Santé (OMS),
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